Dr.Tracker Outreach

System overview — architecture, features, API access, and roadmap

1. What this is

A browser-based tool that helps Dr.Tracker reach specialists and clinic doctors across India by email — building a contact list, generating personalised outreach drafts with AI, and (once connected) sending and tracking them, all from a single private page. Built for India specifically: IST-aware send timing, Hindi/Hinglish/English drafting, and Indian data-protection (DPDP Act, 2023) compliance rather than US rules.

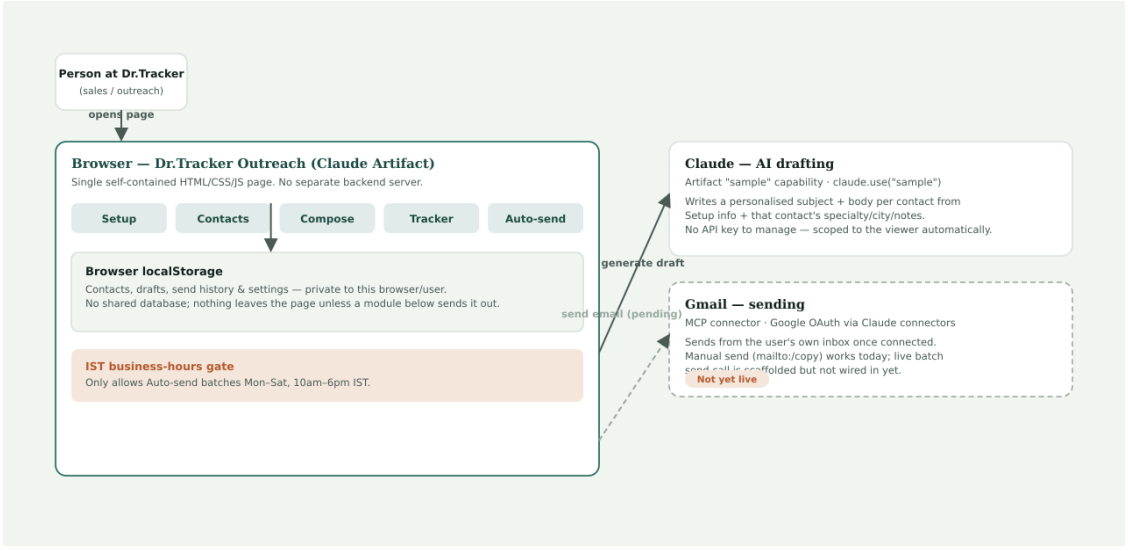
2. Architecture

Client-only web app — no separate backend server was built. Everything runs in one HTML/CSS/JavaScript page, published as a private Claude Artifact.

- Data layer: contacts, drafts, settings and send history are stored in the browser's localStorage, private to the person who opens the page. Nothing is sent to a shared database.
- Drafting: the page calls Claude (the platform's "sample" capability, model-agnostic — no API key to manage) to write a personalised subject + body per contact, using the Setup info and that contact's specialty/city/notes.
- Sending: designed to call the Gmail connector (Google's MCP integration, via OAuth through Claude's connector system) to actually send mail. This piece is scaffolded in the UI but not wired to a live send call yet — see Section 4.
- Scheduling: a client-side IST clock gates auto-send to 10am–6pm, Monday–Saturday, so batches don't fire at times Indian clinics won't read them.

Flow:

- Setup → Contacts → Compose (AI draft) → Send (manual today) / Auto-send (batch, pending Gmail) → Tracker



How a request moves through the system — browser page, local storage, and the two external calls.

3. Implemented today

Area	What it does
Setup	Sender identity, tone, language (English/Hinglish/Hindi), and a separate pitch + value props for each of the 4 contact types, plus the compliance address.
Contacts	Each contact is typed as Doctor, Hospital, Company, or Product partner — this drives which pitch and prompt Compose uses. Add one-by-one or paste from a spreadsheet. Duplicate emails are blocked automatically.
Compose	One-click AI draft per contact, using the pitch that matches its type, fully editable. Copy-to-clipboard or open-in-mail-app (mailto:) to send by hand right now.
Compliance	DPDP Act–oriented footer (identity, address, opt-out) auto-appended at send time; can't be edited out of the draft by accident.

Tracker	Funnel view (not started → drafted → sent → replied → adopted) and a status table per contact.
Auto-send (partial)	Queue view, batch size and delay controls, IST sending window — UI and logic are ready; the live Gmail send call itself is not wired in yet.

4. API keys / access required

No API keys need to be generated or pasted anywhere — both dependencies are handled through Claude's own connector and platform system, not raw keys stored in the app.

Dependency	Used for	How access works
Claude (AI drafting)	Writing the personalised subject + body for each contact.	Built into the Artifact platform ("sample" capability). No key — access is scoped automatically to whoever opens the page.
Gmail (sending)	Actually sending / auto-sending email from the user's own inbox.	Google OAuth via Claude's connector system — the user connects their Gmail account once from the chat; not yet finished wiring on the send call itself.

5. Not implemented yet — and what to build next

- Live Gmail send call — the queue and throttling exist; the actual send action needs to be built and tested against a real connected inbox before it goes live.
- Reply / bounce detection — currently status changes are manual. Real automation needs inbox-read access to detect replies and bounces and update status itself.
- List sourcing — the tool has no way to find doctors' emails; that has to be supplied by the user from a legitimate, consent-respecting source.
- Deliverability infrastructure — a personal Gmail inbox is fine at low volume; real scale (100s/day) needs a proper sending domain with SPF/DKIM/DMARC, ideally through a transactional email provider.
- Content QA — AI drafts aren't fact-checked against anything beyond what's typed into Setup; a human skim before wide sending is recommended, at least initially.
- True background automation — this is a browser page, not a server job. It can only send while it's open; a always-on "set and forget" sender would need a real backend/cron, which is a separate build.
- Multi-user / team use — currently single-user (localStorage, one browser). A shared team version would need the platform's shared database ("db") capability instead.

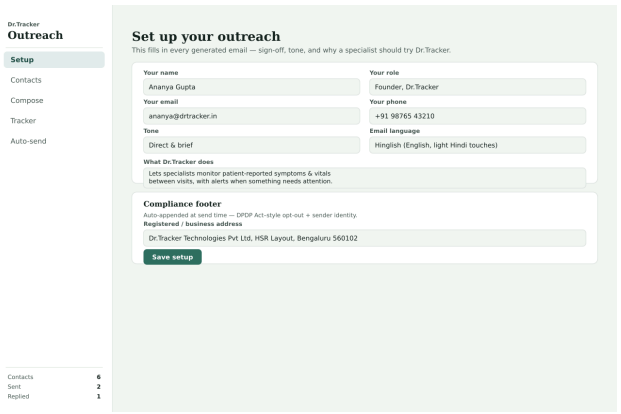
6. How it works, end to end

- 1. Fill in Setup once — who's sending, what Dr.Tracker does, why a doctor should care, the compliance address.
- 2. Add contacts — manually or pasted from a sheet; duplicates are rejected.
- 3. Open Compose, pick a contact, generate a draft — Claude writes it using Setup + that contact's details.
- 4. Review/edit, then send via mail app or copy — the compliance footer is added automatically.
- 5. Status updates on the Tracker as contacts move through the funnel.
- 6. Once Gmail is connected and the send call finished: Auto-send can queue eligible contacts and fire a whole batch, throttled and restricted to IST business hours.

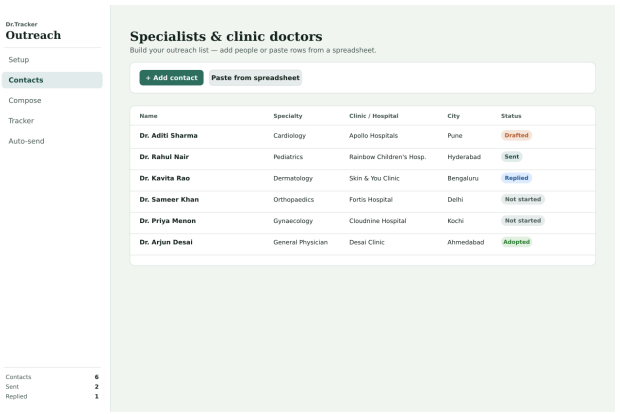
Live artifact: https://claude.ai/artifact/Ja2qZazSZTvXUfJUkpebpv

7. Screenshots

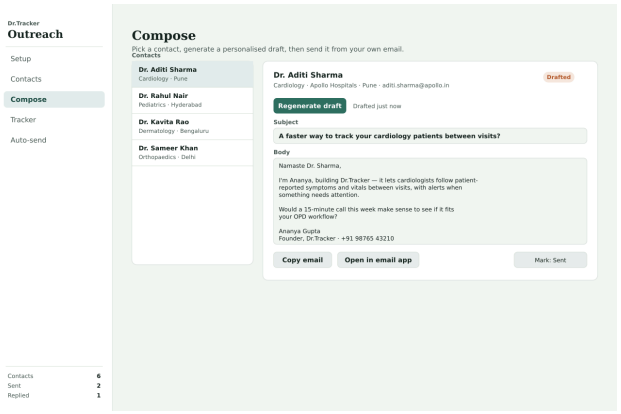
Mockups redrawn to match the live tool's actual layout and design tokens exactly (colours, type, spacing) — not live browser captures, since this environment can't run a headless browser. Sample data shown for illustration.



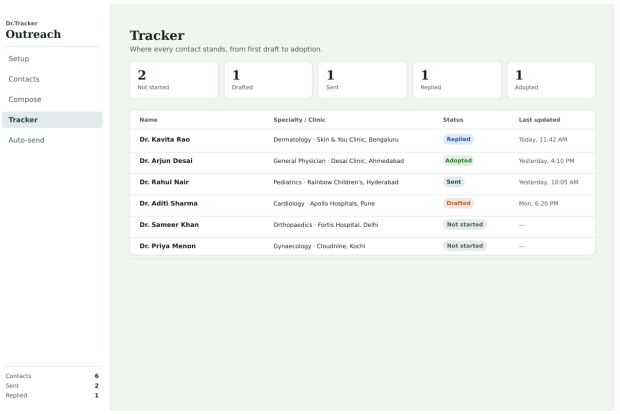
Setup — sender identity, tone, language, product pitch



Contacts — list with specialty, clinic, city, status



Compose — AI draft, editable, ready to send



Tracker — funnel + per-contact status

8. Demo emails

What Compose generates for each contact type — built on a concrete, low-key scenario rather than feature talk, with no invented statistics, testimonials, or fake urgency. Example personas, not researched targets.

DOCTOR — CARDIOLOGIST

Subject: She called three days too late

Namaste Dr. Sharma,

Picture this: a patient leaves your clinic feeling fine. Two days later, something feels off — a little breathless, a little tired. She waits. "It's probably nothing," she tells herself. By the time she calls, it isn't nothing.

You've lived this story more times than you'd like. The space between visits is where things go quiet — and where they can quietly go wrong.

That's the exact gap Dr.Tracker was built to close. Patients check in between appointments — symptoms, vitals, the small stuff they'd normally sit on — and you get a quiet flag the moment something needs your eyes. No new dashboard to babysit. No extra admin. Just a signal, right when it matters.

I'd love to show you what this actually looks like inside a real OPD — 15 minutes, no slides, just a live walkthrough on your phone. When's a good time this week?

Warmly,
Ananya Gupta
Founder, Dr.Tracker · +91 98765 43210

P.S. If a call feels like a big ask right now, just reply "video" and I'll send a 2-minute look instead.

DOCTOR — ORTHOPAEDIC SURGEON

Subject: The swelling she waited four days to mention

Namaste Dr. Nair,

Ten days after a total knee replacement, a patient notices her knee is warmer than it should be. A little more swollen. She tells herself it's normal — recovery is supposed to hurt, right? Four days later, she's back in your OPD with an infection that would've been a five-minute fix if you'd caught it on day one.

Post-op recovery is where orthopaedic care goes quiet. Patients go home, physio compliance slips, and the only way you find out something's wrong is if they call — and by then, it's often no longer simple.

Dr.Tracker gives your post-op patients a simple daily check-in — pain, swelling, mobility, physio adherence — and flags you the moment something looks off. Not a portal they'll ignore. Just the signal you actually need, when you need it.

Would 15 minutes make sense this week to see how it'd sit inside your post-op protocol?

Warmly,
Ananya Gupta
Founder, Dr.Tracker · +91 98765 43210

P.S. If it's easier, reply "video" and I'll send a 2-minute walkthrough first.

HOSPITAL — INSTITUTIONAL PILOT

Subject: What happens to your patients after they leave your wards?

Dear Dr. Malhotra,

A patient is discharged from Navjeevan on a Friday. By Monday, something isn't right — but she doesn't call. Not because she doesn't trust your hospital, but because she assumes it's nothing, or doesn't want to "bother" anyone. By Wednesday, she's back in your ER, and what could've been managed with a phone call is now a readmission.

Every hospital in India carries this cost quietly — in bed turnover, in doctor time spent on manual follow-up calls that mostly go unanswered, and in patients who quietly decide your hospital didn't check on them.

Dr.Tracker gives every doctor across your departments a simple way to stay connected to discharged patients — daily check-ins, vitals, a flag the moment something needs attention — without adding a task to a nurse's or doctor's day. For a hospital like Navjeevan, that's fewer avoidable readmissions, doctors who aren't chasing patients on WhatsApp, and a real answer when NABH or your own leadership asks how you follow up after discharge.

We're piloting this with a small number of hospitals across India right now. Could we get 20 minutes with you and your clinical governance team to walk through what a pilot could look like for Navjeevan?

Best regards,
Ananya Gupta
Founder, Dr.Tracker · +91 98765 43210

P.S. Happy to start with a one-page pilot proposal if that's easier to circulate internally first.

COMPANY — NETWORK PARTNERSHIP

Subject: Who notices when a patient goes quiet?

Dear Ms. Varma,

Right now, somewhere in your network, a patient is sitting with a symptom they're not sure is worth mentioning. Most will wait. A few will wait too long — and when they do, it's not just their health on the line. It's whether they still trust that your network was watching.

That gap between visits is invisible today. Almost every hospital and diagnostic network in India has it. Almost none of them are doing anything about it.

Dr.Tracker gives your affiliated doctors a simple way to stay connected to patients between appointments — quick check-ins, vitals, a gentle alert the moment something needs attention. For a network like Sanjeevani, that's fewer patients slipping through, doctors who feel backed up instead of stretched thin, and a real answer the next time someone asks, "what happens if something goes wrong after I leave?"

We're partnering with a handful of forward-looking networks across India right now, and I'd love Sanjeevani to be one of them. Could we grab 20 minutes with your clinical leadership — I'll bring the specifics, you bring the tough questions?

Best regards,
Ananya Gupta
Founder, Dr.Tracker · +91 98765 43210

P.S. Happy to start with a short async demo video first if that's easier to route internally.

PRODUCT PARTNER — OFFERING OUR DOCTOR NETWORK

Subject: Our doctors are already telling patients to track their vitals — with what?

Dear Ms. Kapoor,

Every week, one of our doctors tells a patient to keep an eye on their blood pressure, their oxygen levels, their heart rate — and every week, that patient goes home and does it with whatever they have lying around, or doesn't do it at all.

We built Dr.Tracker around a simple idea: the gap between a patient's visits shouldn't be a black box. Today, a growing group of specialists — cardiologists, orthopaedic surgeons, gynaecologists — use it to stay connected to patients between appointments. They're exactly the kind of doctors who'd want their patients using a device like VitalSense — accurate, simple, feeding straight into the same care they're already trying to give.

We'd love to explore a partnership: introduce VitalSense to doctors on Dr.Tracker who are already primed to recommend exactly what you've built, and see what adoption looks like when the doctor recommending it is one the patient already trusts.

Would 20 minutes make sense this week to see if there's a fit?

Best regards,
Ananya Gupta
Founder, Dr.Tracker · +91 98765 43210

P.S. Happy to share which specialties on our platform would be the strongest early fit, if that's useful before we talk.